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Build. Transform. Grow.

PREPARED FOR

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Chief Executive Officer
Iwosan Investments / IASO Medipark

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Iwosan and Medipark
The year either side of opening

What tends to get heavy in the twelve months on either side of an opening, what I would want to know before saying anything firmer, and what I could take off your plate.

Not a proposal. No fee in it. Written from the public record, because I have not seen your numbers.



WHERE THE PRESSURE SITS

01 **The ramp**
02 **The bench**
03 **Five addresses**
04 **The empty space**
05 **The payer mix**

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WHY I AM WRITING

The last time we worked on a building together, it had six weeks.

You have known me since I was pitching Doctoora at you, and you watched the rest of it from Evercare through to Cedarcrest. So no introduction. Just the reason for the note.

2019

Doctoora

You saw the raise. It went on to run across 21 states and was acquired by Evercare

2020

Eti-Osa

The isolation centre. 90 beds, a 10-bed ICU and the state reporting dashboard, live in six weeks

2021

Evercare

Chief Innovation and Strategy Officer. USD 1.1m out, volume held, pay for performance in

2024

Cedarcrest

Chief Executive. N1.0bn a month and N1bn H1 EBITDA, the best in the hospital's history

NOW

Consult for Africa

Advisory and venture firm. We take build and operate mandates, so the plans come back to us as a number

Medipark is a different order of thing entirely, but the questions in the months before a facility opens are largely the same questions. So the question I want to ask you is a simple one. What can I take off your plate?

HOW I READ IT

Six things that get heavy between groundbreaking and run rate

None of this is a claim about Iwosan. It is the pattern in facilities at this stage, written so you can test it against what you already know.

01

A 140-bed hospital does not open, it ramps

And under a forty year concession the ramp is the return

02

The scarce input is the consultant bench

Eight to twelve months of lead time, and it is the proposition

03

A fifth address in Ikoyi is a portfolio question

Minutes from your own hospitals, so growth can be transfer

04

The square metres that are not the hospital are businesses

Four or five of them, and they are the ones that get forgotten

05

Twenty per cent of the beds sets the arithmetic for eighty

Payer and tariff architecture has to exist before day one

06

Repatriating medical tourism is a purchasing decision

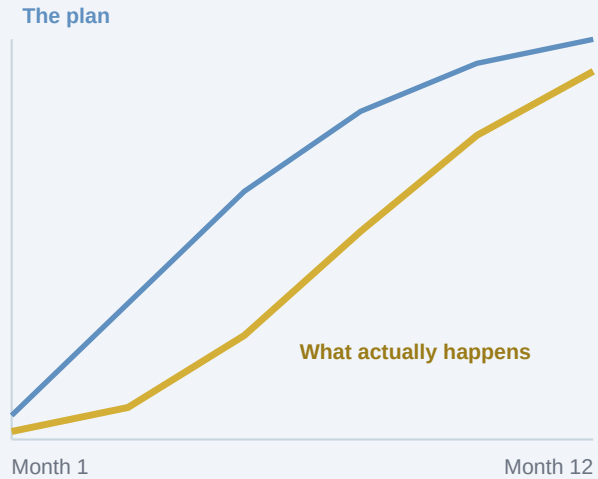
Families buy certainty, which has to be packaged and priced

An opening is a date. A ramp is a curve. Plans confuse the two.

The first ninety days after a hospital takes its first patient go to defect correction, induction, workflow rework and everything the drawings did not anticipate, which moves the revenue curve a quarter to the right while the cost base arrives on time and in full.

- **Why it compounds.** The early years are the ones a forty year concession earns off, so a quarter lost at the start is not a quarter lost once.
- **Why it lingers.** A facility that opens quiet acquires a reputation for being quiet, and that takes a year of marketing to undo.
- **The answer is not to open faster.** It is to open in a deliberate sequence, service line by service line, with the monthly ramp modelled and one person accountable to the number rather than to the date.

THE SHAPE OF THE FIRST YEAR



The gap between the two lines is funded by somebody. Usually the shareholder.

The scarce input is the consultant bench, not the capital

A multi-specialist hospital of this size needs a named consultant in every service line that carries margin. Counting notice periods, credentialling and relocation, that is a recruitment problem with eight to twelve months of lead time.

Leave it to the construction timetable

You open on locum and visiting cover in exactly the specialties where continuity is what the patient is paying for.

The first six months set the reputation

Referring doctors decide who works there early, and that judgement is expensive and slow to correct afterwards.

The bench is the proposition

If Medipark is meant to keep Nigerian patients in Nigeria, the names on the rota are the product, not the equipment.

Senior Nigerians practising abroad have to be worked as a real recruitment channel, with terms, sessions and a landing plan, rather than as a line in a launch announcement.

Awolowo Road is minutes from two hospitals you already own

Which means the decision that matters is not what Medipark can do. It is what each of the five addresses is now for.

Ikeja	Apapa	Victoria Island	Ikoyi	Medipark
Iwosan Lagoon	Iwosan Lagoon	Iwosan Lagoon	Iwosan Lagoon	140 beds, opening

WITHOUT AN EXPLICIT POSITION, THREE THINGS HAPPEN

- The new asset gets priced as a better version of a hospital you already own, because the existing schedule is the nearest reference point anybody has.
- The same consultants split their sessions across two of your buildings, and neither rota is full.
- A share of the volume that arrives looks like growth while it is actually transfer, which shows up first in the Ikoyi and Victoria Island books.

THREE DECISIONS, BEST MADE ON PAPER

- Which service lines sit where
- How referral flows between sites are designed and rewarded
- Who owns the profit and loss for a patient who could go to either

Four or five businesses sit on that campus besides the hospital

Each one has a tenant or an operator, a tariff, an occupancy assumption and somebody whose job it is to fill it. What happens in practice is that the hospital absorbs every hour of management attention in the opening year, because the hospital is the thing that can hurt somebody, and the rest of the floor plate sits empty for eighteen months while carrying its share of the cost.

- **Diagnostics.** Its own referral base, its own price list, and outside demand from the moment it is accredited.
- **Training.** A school, a continuing education business, a recruitment funnel, or all three. Each earns differently.
- **Retail medical.** A rent roll, with a tenant mix that either feeds the hospital or competes with it.
- **Residential.** The most forgiving line on the campus and usually the first one paying.

FROM A CAMPUS WE STRUCTURED LAST YEAR

~1/3

of stabilised campus revenue came from the lines that were not the anchor clinical service.

And the fastest of them was paying before the anchor service was. Which is the whole argument for giving each line a profit and loss, an occupancy target and a name against it well before the doors open.

Twenty per cent of the beds sets the arithmetic for the other eighty

Reserving a fifth of the beds for indigent patients is the right thing to have negotiated, and it is also the most demanding piece of commercial design in the project, because the remaining beds carry a fixed cost base at an occupancy and a price that a concession has already shaped.

What has to be settled before day one	The question inside it
The self-pay proposition	Why a family chooses you at your price, said in a sentence that is not about equipment
Corporate, embassy and international accounts	Which are signed rather than discussed, because contracting takes longer than commissioning
The managed care position	Which schemes you take, at what tariff, and where you are willing to refuse
The reserved beds	Funded, reported and staffed as a programme with its own accountability, rather than absorbed as a loss and quietly resented by the people delivering it
Medical tourism pathways	Three or four packaged end to end at one price, consultant named, escalation written down, live on day one. Families are buying certainty, not capability

Conditionals, because I have not seen your numbers

If	Then expect	What I would do about it now
Senior recruitment runs on the construction timetable	Opening on locum cover in the specialties that carry margin, and a first year spent buying back a reputation	Run the bench now as a named-seat exercise by specialty, diaspora included, with assessment on every senior hire
The campus is underwritten as one building	Eighteen months of empty non-clinical floor plate carrying its share of the fixed cost	Give each line its own profit and loss, occupancy target and named owner before opening
Tariffs are set against the existing Lagoon schedule	The new asset reads as a better version of a hospital you own, and volume is transfer rather than growth	Price the campus separately, with a written position on what each of the five addresses is for
The opening is planned as a single date	The first ninety days go to defects and induction, and the first-year miss is blamed on demand	Sequence by service line, high certainty first, and model the ramp monthly with one owner
Medical tourism is positioned as a capability	Enquiries in volume and conversions in single figures	Package three or four pathways end to end, one price, named consultant, written escalation
Support functions are shared with the group without a settled operating model	The best people at Lagoon get pulled into Medipark and four hospitals soften at the wrong moment	Settle the group operating model first: what consolidates, what stays local, who backfills

The seven questions that would change everything I have just said

I would rather tell you what I cannot see than write around it. Each of these moves my view materially, and each is answerable in a conversation rather than a study.

	The question I would ask first
The ramp	What the commissioning plan assumes month by month for year one, whether it is modelled or asserted, and who personally owns that number
The bench	Where senior recruitment stands by specialty, counted in signed terms rather than expressions of interest
The rent roll	Whether the non-clinical space has an occupancy assumption, a pricing basis and an accountable owner, or is still described as future upside
The payer position	The strategy for the eighty per cent, and which corporate, embassy and international accounts are signed rather than in discussion
The group	How the four existing hospitals change once the fifth address opens, and whether that is written down anywhere the management teams can read it
The academy	What the training facility is expected to earn, and whether it is a school, a CPD business, a recruitment funnel or all three
The consortium	How delivery responsibility divides between the partners once the building is finished, because operating asks for a different structure from building

WHERE I COULD BE USEFUL

Six shapes of help. Take one, or none.

Pieces of work rather than a programme. Each one stands alone, and each one starts as two or three weeks with a fixed scope.

Commissioning and activation

A sequenced opening plan service line by service line, the ramp modelled monthly, readiness gates defined, and the first ninety days planned as an operating period rather than a launch

The consultant bench

Named-seat recruitment in the specialties that carry margin, diaspora included, structured assessment on every senior appointment, and pay that survives the second year

Campus economics

Diagnostics, training, retail and residential run as businesses with tenants, tariffs, occupancy targets and owners. This is the work we did on a campus of comparable ambition

Payer and tariff architecture

Price positions by site and service line, the self-pay proposition, corporate and international contracting, and the funding design for the reserved beds

Group operating model

What sits where across five addresses, how referral flows are designed and rewarded, where support functions consolidate, and how the existing hospitals are protected

Fractional or seconded leadership

If what is missing is a pair of experienced hands inside the building rather than a view from outside it. We second executives on fixed terms and hold the operating result

The part of the story you will not know

A healthcare advisory and venture firm in Lagos and Abuja, built by operators. We take build and operate mandates as well as advisory ones, which means the plans we write come back to us as a number we have to hit. I run it alongside the health technology companies I founded.

WHAT WE DO

- Turnaround and financial recovery
- Strategy, growth and commercial performance
- Build, commission and operate
- Clinical governance, quality and accreditation
- Workforce, senior recruitment and leadership
- Fractional leadership and executive secondments
- Digital, hospital systems and decision reporting

Senior recruitment runs off a verified registry of more than ten thousand Nigerian clinicians that we build and maintain in house.

SELECTED RESULTS

N1.0bn a month and N1bn H1 EBITDA at Cedarcrest Abuja, under me as chief executive. The best year in its history

USD 1.1m of annualised cost out at Evercare Lekki, with clinical volume held

N1.7bn medical campus structured last year: measured geometry, rent roll, monthly cash model, peak funding and payback

N2bn+ of planned annual revenue underwritten since the firm was founded, across five health businesses

Client names withheld where the work is live. References on request.

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THE ASK

A conversation, not a mandate.
Tell me what is heaviest this month.

I will tell you honestly whether it is something we should be doing for you or whether you have it covered already. If the timing is wrong, say so and I will come back after you have opened, because the second year of a campus asks harder questions than the first and I would be glad to be useful then instead.

Either way, I would love to see the place before it opens.



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